

An Autistic Worker in Your Occupational-Health Team

For occupational-health nurses & workplace health staff — supporting an autistic adult in employment.

The one idea

Autism, through **monotropism**, means an adult's attention locks into a few deep, intense "tunnels." Workplaces — open-plan, noisy, full of interruption and unwritten rules — drive masking and **autistic burnout**, which is distinct from ordinary occupational stress and depression. As an OH nurse you're well placed to spot it, separate it from performance issues, and broker the reasonable adjustments that keep a capable worker well and in work.

What you may see — and what's really going on

You may see...	What's often happening
Exhausted, losing skills, can't recover after work	Autistic burnout — distinct from occupational burnout and depression
"Fine" in clinic, collapses for days after	Heavy masking ; a 15-minute visit rarely shows the cost
Doesn't report pain, symptoms or side-effects until severe	Interoception differences
Clashes linked to open-plan, meetings, hot-desking	An environment-fit problem, not a competence problem
Wants everything in writing; avoids calls	Single-channel communication reduces the load

Try this

- **Offer the AASPIRE AHAT** pre-visit and **written-first** options; allow a supporter.
- **Recognise autistic burnout** and don't confuse it with depression or "poor resilience."
- **Broker reasonable adjustments:** quiet space, written instructions, fewer meetings, asynchronous communication, defined expectations.
- **Start medications low and titrate slowly** — atypical sensory/emotional responses are common.
- **Document effective adjustments** so they are automatic next time.

Avoid this

- Letting a calm, masked visit override self-report and history.
- Treating burnout as depression and pushing a "return-to-full-load" plan.
- Equating "not complaining" with "not suffering."

When to get more support

Assess for **suicidality** — burnout is linked to it. Address co-occurring **ADHD (AuDHD)**, **anxiety**, **depression**, **sleep** and **GI conditions**. Where burnout is severe, validate phased return, reduced hours or a role change as recovery, not weakness.

If the worker is a woman

Late-diagnosed women mask most heavily; take a lifelong pattern of difference and overload seriously, even with a “high-performing” exterior.

About this guide

*AI-assisted, derived from this project's research drafts — the **Lifespan Practitioner & Health-Care guide** (Parts 3.4 & 4) and the **Monotropism** brief. Uses identity-first language. **Informational — not medical advice or a diagnostic tool.** Fit it to the individual.*

Key sources. Murray, Lesser & Lawson (2005); Murray, F. (2023); Dwyer (2025); Raymaker et al. (2020); Mantzalas et al. (2022); Kelly Mahler (2024); AASPIRE AHAT. Full citations are in the source drafts.